

TOOLBOX TALK

Drugs and alcohol at work

Delivery script for the person running the talk

Organisation: {Company name}

Presenter: {Presenter}

Date of talk: {Date}

Policy version referenced: {Policy version}

This booklet carries the words. The slide deck carries the pictures, and the speaker notes on each slide are only short cues. Read from this booklet if that is easier — nobody minds.

What you need on the day

- The slide deck, on a screen everyone can see
- Sound, tested before people arrive — there is a short film in slide 7
- The printed sign-off sheet, and a pen that works
- This booklet

v1.1 — issued 1 September 2026. Prepared with NIVHA Laboratory Services. A template for the organisation to review and adopt — it is not legal advice and it is not a substitute for the policy itself.

Before you start

You are not expected to be an expert on drugs, alcohol or testing. You are expected to read this out clearly, keep the room calm, and know what to do with a question you cannot answer. That last part is on the next panel, and it is the most important page in this booklet.

Your versions of two slides

- Two slides come in three versions — Northern Ireland, Republic of Ireland and Great Britain — each labelled in the top right corner.
- Keep the one that matches where you operate, delete the other two of each, then delete the instructions slide at the front.
- This booklet carries the words for all three versions, labelled the same way. Slide numbers refer to the finished 14-slide deck, after those deletions.

The room

- Somewhere people can sit and see the screen. A canteen out of hours works; a noisy workshop does not.
- Turn phones face down rather than banning them — people will want to photograph the support slide.
- Test the film on slide 7 with the sound on, before anyone arrives. If there is no internet, use the four moments listed on the slide instead and describe them.
- Print the sign-off sheet. Everyone present signs it, including you.

The timing

The core talk runs about 25 to 35 minutes, including the film and the three questions at the end. The timings against each section are a guide, not a target. If the room is quiet, you will be at the shorter end; if people are asking questions, take the longer end and let them.

The tone

- Safety, not surveillance. Say that early and mean it.
- Nobody is in trouble, and this is not an investigation.
- Ask people not to use anyone by name as an example, including themselves. Situations, not people.
- If someone becomes upset, move on and offer a private word afterwards. Do not make it a discussion.

The one rule for questions

If you are asked anything you cannot answer — about the science, about someone's medication, about what would happen in a particular case — **park it**. Say: "That is a fair question and I am not going to guess at it. I will write it down and get you a proper answer."

Write the question in the **parked questions** box on the sign-off sheet and pass the sheet to (Policy owner) after the talk. They resolve it against the policy, or through the organisation's testing provider arrangements, and the answer comes back to whoever asked.

Never guess. A confident wrong answer about testing is the single most damaging thing that can happen in one of these sessions.

The talk, slide by slide

Words in (angle brackets) are filled in from your policy before the talk. Read the shaded text aloud. The prompts underneath are for you.

Slide 1 · about 1 minute

Cover — welcome

SAY THIS

Morning, everyone. Thanks for stopping. This is a short toolbox talk about drugs and alcohol at work, and it will take about half an hour.

Before we start, two things. First, nobody is in trouble and this is not an investigation — it is a briefing, the same as any other safety briefing we do. Second, I am going to show you exactly how a test works, step by step, so that if you are ever asked to do one, you already know what happens.

I did not write this and I am not an expert on it. It was put together with NIVHA Laboratory Services, who do the testing. If you ask me something I do not know, I will write it down and get you a proper answer afterwards rather than make one up.

DO

- Have the sign-off sheet out where people can see it. Say it goes round at the end.

Slide 2 · about 2 minutes

Why we are talking about this

SAY THIS

Here is what this is not. It is not about your weekend. It is not about your private life or what anybody does on their own time. Nobody is policing that and nobody wants to.

The policy asks one question, and it is a narrow one: are you fit to do your work safely today. If the answer is yes, the policy never touches you. If the answer is no, the policy is the thing that stops you getting hurt.

There is a duty on both sides here. The company has to protect everyone at work, so far as is reasonably practicable. And each of us has a duty not to be under the influence of anything to the point where we put ourselves or somebody else in danger.

The other half of it is support, and I will come back to that near the end. Almost everybody who ends up in trouble over drink or drugs at work knew there was a problem beforehand and said nothing. There is a door in this policy for that moment.

ASK

- Which three jobs on this site would go worst wrong if someone lost concentration for a second? Take a few answers and keep them in mind — you will refer back.

Slide 3 · about 2 minutes

The scale of it

This slide comes in three versions. Read only the one you kept in the deck.

Northern Ireland version — read this if you kept the NI slide

SAY THIS

I want to give you some numbers, because most people think this is somebody else's problem, somewhere else.

In Northern Ireland, two hundred and fifty-one drug-related deaths were registered in 2024, and the great majority of those met the definition of drug misuse.

The single drug named most often was pregabalin, which is a prescription medicine, on just over a hundred of those certificates. That one is worth pausing on, because it breaks the assumption that this is only about something bought on a street corner.

Opioids as a group appeared most often overall, and the highest rate was in the thirty-five to forty-four age group.

These are not figures from a campaign group. They are from the statistics agency. And these are working-age people — people with jobs, in workplaces like this one.

Republic of Ireland version — read this if you kept the ROI slide

SAY THIS

I want to give you some numbers, because most people think this is somebody else's problem, somewhere else.

In Ireland, three hundred and forty-three drug poisoning deaths were recorded in 2022, which was the second year running that the figure fell.

Cocaine was implicated in about a third of them. And cocaine poisoning deaths rose by two hundred and fifty-nine per cent between 2013 and 2022, the largest rise of any drug group.

These are not figures from a campaign group. They come from the Health Research Board, which examines every case rather than sampling. And these are working-age people — people with jobs, in workplaces like this one.

Great Britain version — read this if you kept the GB slide

SAY THIS

I want to give you some numbers, because most people think this is somebody else's problem, somewhere else.

In England and Wales, five and a half thousand deaths related to drug poisoning were registered in 2024. That is the highest number since these figures began in 1993.

Getting on for thirteen hundred of those involved cocaine, and that was a rise of more than fourteen per cent in a single year. The rate has gone up every year since 2012.

These are not figures from a campaign group. They are from the statistics agency. And these are working-age people — people with jobs, in workplaces like this one.

DO

- Read the numbers, let them land, and move on. Do not dwell. Somebody in the room may have lost a person this way.

Slide 4 · about 2 minutes**What counts****SAY THIS**

When the policy says drugs and alcohol, it means four things, and the last two catch people out.

Alcohol — during working hours and paid breaks, and bear in mind it can still be affecting you the morning after a heavy night.

Illegal drugs — cocaine is the one found most often in workplace samples now, with cannabis second.

Prescription and pharmacy medicines — only where they are misused, or where something that affects your alertness has not been declared. Strong painkillers, some cough medicines, sedating antihistamines, sleeping tablets. The test does not know why something is in your system. Declaring is what supplies the why.

And psychoactive substances — the things sold as legal highs. Nobody taking them knows what is actually in them, which is exactly the problem.

To be clear: a prescribed medicine that you have declared, handled properly, is not a breach of the policy. An undeclared one is a different conversation.

Slide 5 · about 2 minutes**Effects at work****SAY THIS**

You do not need any science for this part. There are four things that go, and every one of them matters on this site.

Judgement goes first. Risks look smaller than they are and shortcuts start to feel reasonable.

Reaction time stretches — the gap between seeing something and doing something about it. Around moving plant, that gap is the whole margin of safety.

Coordination and balance slip. Grip, footing, fine control. That shows up fast on a ladder or with a tool in your hand.

And attention drifts, with tiredness that can last well past the effect itself.

Here is the part worth remembering: the person affected is almost always the last one to notice. That is why the standard is fitness for work, and not just how you reckon you feel.

ASK

- Think about the three jobs we named earlier. Which of those four would you least want to lose on that task?

Slide 6 · about 3 minutes**Our policy, the rules here****SAY THIS**

This is our own document, so let me tell you what it actually says.

The standard is fitness for work. Not abstinence in your private life — fitness for work when you are here. During working hours and paid breaks there is no alcohol and no drugs, and if a substance could be affecting you, you do not start.

It applies to everyone on site, and that includes contractors and visitors. If you supervise contractors, that is worth knowing.

The testing that applies here is (Testing types in your policy).

The policy is strict about breaches and I am not going to soften that. What I will say is that the process before any of that happens is the one I am about to walk you through, and it is built so that decisions rest on evidence.

(Policy owner) owns the policy and the full document is at (Where the policy is published). Read it once today rather than for the first time on a bad day.

DO

- Read out only the testing types that actually apply here. If there is no testing programme, say so plainly and say that people would be consulted first if that ever changed.

Slide 7 · about 5 minutes including the film**How testing works — the film****SAY THIS**

Rather than describe this, I am going to show you. This is a short film of a mouth swab collection from start to finish.

Before I play it, watch for four moments: the sealed kit being opened in front of the person, how the swab is handled and by whom, the tamper-evident seal going on, and the paperwork being checked and signed. That is the whole procedure.

Nobody in the film is an employee of any client. It is a demonstration.

[Play the film.]

That paper trail has a name — chain of custody — and it sounds like jargon, but it is the best protection you have. From the moment a sample leaves your body there is a record of who had it, when, and what they did with it. It means nobody can suggest your sample got mixed up with somebody else's or interfered with. And if any link in that chain is broken, the result is not defensible and cannot be used.

You check the details, you sign, and you keep a copy.

If the screen is negative, that is the end of it and you go back to work.

ASK

- Was anything in that different from what you expected? Most people expect something far more intrusive, and it is worth letting that relief happen.

Slide 8 · about 3 minutes

Screening and confirmation

SAY THIS

This next part is the most misunderstood thing about the whole subject, and getting it straight might save somebody here a very bad week.

The device used on site is a screening device. It gives an indication. It does not identify a substance definitively and it is not evidence. That is why we do not use the word positive at that stage. The word is non-negative.

A non-negative screen means one thing: further samples go to a laboratory. Nothing more than that.

Confirmation is a completely different activity, done by a laboratory, using different methods, on separate samples, against that laboratory's own current published cut-off schedule. That is the evidential step, and that is what produces a certificate.

While that is happening, you are stood down from safety-critical work. That is a precaution, not a punishment and not a finding. A further sample is kept so that independent re-analysis can be asked for if the result is disputed.

So, once more, because it matters: a non-negative screen is not a failed test. It is the point where the sample goes to a laboratory.

DO

- Do not offer any numbers here — not levels, not timings. If you are asked, the honest answer is that the analysing laboratory publishes its own schedule and interprets the result against it. Park anything further.

Slide 9 · about 3 minutes**Medication****SAY THIS**

Three practical points, and for most people in this room these are the most useful three minutes of the talk.

One: both prescribed medicines and things you buy at a pharmacy count. Codeine-containing painkillers, some cough medicines, sedating antihistamines, sleeping tablets, prescribed sedatives. The screening panel does not know why something is there.

Two: declare it before, not after. Before a test, and before your first shift on something new. The confidential route is (Medical declaration route), and you use that rather than telling me what condition you have. You never have to tell your line manager your diagnosis. Declared beforehand, a finding that matches your prescription is explained before anybody could misread it. Declared afterwards, it looks like an excuse produced to fit a result, and that is a much harder road for you.

Three: read the label. If it says may cause drowsiness, or do not drive, or do not operate machinery, that warning is about exactly the work some of you do. Tell somebody so the task can be looked at.

On prescribed cannabis-based products, because it comes up every time: they are medication and they go through the same route. A clinician looks at the laboratory finding alongside your declared prescription. What I am not going to tell you — and what you should not believe from anyone — is that a prescription means nothing will be found. People have got themselves into real difficulty believing that.

And CBD products are not tightly regulated. Some contain more than the label claims. If you use one, declare it the same way.

ASK

- Has anyone here taken something over the counter that carried a drowsiness warning? Antihistamines, cold and flu remedies, strong painkillers — all ordinary, and none of it anybody's fault.

Slide 10 · about 2 minutes**Your rights****SAY THIS**

Four things you are entitled to, and this is the part people are most anxious about and least often told.

Consent. A sample is given with your agreement, after the procedure has been explained. You can ask for it to be explained again at any point, and you can ask questions throughout.

Dignity. A private room is used — not the canteen, not the shop floor. Nobody stands over you in a way that would embarrass you.

Confidentiality. A result is health information, and it has stronger protection than ordinary records. It goes to the authorised contacts named in the policy and to nobody else. If I am your supervisor, I am told whether you are fit for work. I am not told what was found. Nothing gets announced or discussed on the floor.

And your own information. You can ask what is held about you, ask for a copy, and ask for anything inaccurate to be corrected. Records are kept only for as long as the policy says, and then disposed of securely.

Slide 11 · about 2 minutes

Refusal and tampering

SAY THIS

One short section, said once and calmly, because it is better known today than discovered on the day.

Refusing to give a sample without good reason is generally treated the same as a positive result. So is walking away, so is spinning it out until the collector has to leave, and so is going home when you have been asked to stay.

Substituting a sample, watering it down or adding anything to it is treated as tampering, and it is the attempt that is the issue. The kits and the laboratory checks are designed to show that up.

If there is a genuine reason you cannot provide a sample, say so at the time so it can be written down. That is a completely different thing from a refusal.

Nobody is being asked to prove they are innocent here. The whole process exists so that decisions rest on evidence rather than somebody's opinion on site.

Slide 12 · about 2 minutes

Support

This slide comes in three versions. Read only the one you kept in the deck.

Slide 12 · Northern Ireland version — read this if you kept the NI slide

SAY THIS

This is the slide to photograph, and I will give you a moment to do it.

If drink or drugs are becoming a problem, say so before it surfaces some other way. Coming forward yourself, before you are ever selected for a test, is treated as a health matter with support — not as a disciplinary matter. Being found out and coming forward are two different things, and the policy knows the difference.

Inside here, the confidential route is {Support contact}, and where an employee assistance programme is in place it is free and it does not go through your line manager.

Outside, your GP can treat this as the health matter it is, and that involves your employer not at all. There are also the services listed on nidirect, and the addiction services run by Inspire. Both are open to anybody, at any stage.

Asking for help is not a breach of this policy. Nothing said today should leave anybody thinking the safe move is silence.

Slide 12 · Republic of Ireland version — read this if you kept the ROI slide

SAY THIS

This is the slide to photograph, and I will give you a moment to do it.

If drink or drugs are becoming a problem, say so before it surfaces some other way. Coming forward yourself, before you are ever selected for a test, is treated as a health matter with support — not as a disciplinary matter. Being found out and coming forward are two different things, and the policy knows the difference.

Inside here, the confidential route is {Support contact}, and where an employee assistance programme is in place it is free and it does not go through your line manager.

Outside, your GP can treat this as the health matter it is, and that involves your employer not at all. There is also the HSE Drugs and Alcohol Helpline, which is open to anybody, at any stage.

Asking for help is not a breach of this policy. Nothing said today should leave anybody thinking the safe move is silence.

Slide 12 · Great Britain version — read this if you kept the GB slide**SAY THIS**

This is the slide to photograph, and I will give you a moment to do it.

If drink or drugs are becoming a problem, say so before it surfaces some other way. Coming forward yourself, before you are ever selected for a test, is treated as a health matter with support — not as a disciplinary matter. Being found out and coming forward are two different things, and the policy knows the difference.

Inside here, the confidential route is (Support contact), and where an employee assistance programme is in place it is free and it does not go through your line manager.

Outside, your GP can treat this as the health matter it is, and that involves your employer not at all. There are also the national services — FRANK for drugs and Drinkaware for alcohol. Look up their current contact details before the session so that you can give them out.

Asking for help is not a breach of this policy. Nothing said today should leave anybody thinking the safe move is silence.

DO

- Pause and let people photograph the slide. Then say the last line again.

Slide 13 · about 2 minutes**Knowledge check****SAY THIS**

Three quick questions before we finish. Hands up, nothing is written down and nothing is scored.

One. What single question is the policy asking about you?

— Whether you are fit to do your work safely today.

Two. What does a non-negative screening result mean?

— That the sample goes to a laboratory for confirmation. It is not a positive and it is not a fail.

Three. When do you declare medication, and what do you not have to say?

— Before a test, and before your first shift on anything new that carries a drowsiness or machinery warning. You never have to say what the condition is.

DO

- Give the answer straight after each question rather than letting the room guess for long.

Slide 14 · about 2 minutes

Close

SAY THIS

If you forget everything else, keep three things.

One: the policy asks one question, and it is about today, not about your private life.

Two: a screening device indicates, a laboratory confirms, and a clinician reviews declared medication. Nothing about you gets decided on one device in a room.

Three: declare your medication and ask for help early. Both of those are ordinary things to do.

And the sentence this whole talk exists for: everyone who comes to work healthy and well goes home the same way. That is the standard, and it applies to every person here, including me.

The sign-off sheet is going round now. Print your name, sign it, date it. It records that the policy was talked through with you, nothing more. If you asked something I could not answer, it is written on that sheet and it goes to {Policy owner}, who will come back to you.

DO

- Pass the sheet round and stay behind for a few minutes for anyone who wants a private word.

Crib sheet — the five questions you will be asked

These five come up in almost every session. Safe answers are below. Anything that goes further belongs on the sign-off sheet, not in an answer you invent.

1. “I have a prescription for medicinal cannabis. Where does that leave me?”

SAY THIS

It is medication, and it goes through the same route as any other prescription: declared confidentially in advance, then assessed through medical review, where a clinician looks at the laboratory finding alongside your declared prescription. Declared and reviewed properly, that is not a policy breach.

Keep in mind: Never say a prescription means it will not show up. That is wrong and people have got into real difficulty believing it. Separately, the safety question stands on its own — if a medicine affects attention or coordination, the task may need to change.

2. “What about CBD? That is legal, isn’t it.”

SAY THIS

CBD products are not tightly regulated, and some have been found to contain more than the label claims, including THC. That is why you declare it through the same confidential route.

Keep in mind: Do not get drawn into what is or is not legal to buy. The point for us is the declaration, not the shop.

3. “I am on prescribed medication. Am I in trouble?”

SAY THIS

No. A declared, prescribed medicine handled through medical review is not a disciplinary matter. You declare it confidentially, you do not have to tell your line manager the condition, and the clinical detail goes to the reviewing clinician rather than to management.

Keep in mind: The one thing that matters is the order: declared before, not explained after.

4. “How long does it stay in your system?”

SAY THIS

It varies — by substance, by the person, and by how much and how often. That is why nobody sensible gives a figure. A result is interpreted by the analysing laboratory against its own current published cut-off schedule, and it is the laboratory that does the interpreting.

Keep in mind: Give no figures of any kind here. If they push, park it on the sheet.

5. “What if I was drinking the night before?”

SAY THIS

Alcohol can still be present the next morning, and people routinely underestimate that. If you are in any doubt about whether you are fit, do not drive and do not start — say so instead. Nobody is disciplined for saying they are not fit before a shift begins.

Keep in mind: No maths, no rules of thumb, no figures. The answer is the doubt test, not a calculation.

After the talk

Four things, and then you are finished.

1. **Check the sign-off sheet is complete.** Every person present should have printed their name and signed, including you. Fill in the site, the date and the policy version at the top.
2. **File it.** The completed sheet goes to {Policy owner} and is kept as the record that the policy was communicated. It records attendance, not a score.
3. **Pass on the parked questions.** Anything written in the parked questions box goes to {Policy owner} with the sheet. They resolve each one against the policy, or through the organisation's testing provider arrangements, and the answer goes back to whoever asked — by email to the team, or at the next briefing.
4. **Follow anything up quietly.** If somebody asked for a private word, or asked for the support contact, deal with that the same day and keep it to yourself.

Repeating the talk

- Run it again for anyone who was not there — nights, holidays, long-term absence — and for new starters as part of induction.
- Refresh it periodically, and always when the policy changes. Record the new policy version on the new sheet.
- If your policy or your testing arrangements change, ask {Policy owner} for the updated deck before running it again.

Where questions go

Questions raised in the session go to {Policy owner}, who resolves them against the policy or through the organisation's testing provider arrangements. Problems with the pack files themselves: NIVHA Laboratory Services · info@nivha.net · 02890 737942.

Supports the {Company name} drug and alcohol policy, version {Policy version}.